

Sick Children and medication policy

Parents/ Carers, should always ring/text extra time on the mornings or afternoons or holiday club, whichever their child/ren are booked in for, if they are going to be off ill so staff are aware of absentees, and whether they have to record and report any specific illnesses. IF you send a text or leave a voicemail, we will always text you back to say we have received your message, if you don't receive such a text, we may not have received your message, so please make sure you do get clarification that we know your child is not going to be attending the session.

If a child is ill when he/she arrives at breakfast club parents/carers will be asked to take their child home again, giving an explanation for this reason.

If a child comes out of class in the afternoon, to attend an afterschool session looking ill, or if the teacher says they haven't been well during the day, staff are well within their rights **not** to accept the child and will ask the school to contact parents, but with consent of the person in charge of the session that day, we could accept the child, (but only if the school staff say parents are going to be a long time, and it's not covid related).

If a child becomes ill during the breakfast club / afterschool club however, a member of staff will contact the child's parents and they will be asked to come and collect their child/ ren, we will make the child comfortable until their parent arrives, by taking them out of the setting into another room (if staffing levels allow this) where a member of staff can look after them until their parent / carer arrives to collect them. If the child worsens whilst waiting to be collected, and the member of staff in charge of the child feels it is in the interest of the child to seek medical help urgently, the parent will be contacted and will be informed that we are seeking medical help.

If , in the above cases, the illness is an infectious one, then the parent/carer will be advised not to bring them to club, until the symptoms have gone, (in the case of covid – negative test results) or medication is started to aid recovery.

Below is a list of infectious diseases, of which extra time will exclude a child from the setting, as we want to keep all of our children as safe and as well as possible whilst in our care, so not to aid any spread of illnesses and / or diseases.

Childhood Illnesses where the child/ren should **not** attend Extra Time are as follows:

Disease/Condition Exclusion period

INFECTION	RECOMMEND PERIOD OF TO BE KEPT AWAY	COMMENTS
Athletes foot	none	Not a serious condition, treatment recommended
Covid-19* oct 23	From 1 st day of symptoms / positive test, at least 3 days then until feeling better to return to club	If a child tests positive, extra time Will not accept the child in club if they have a temperature or showing signs of feeling unwell. They will be allowed back into club when they are feeling better, staff want to keep everyone as safe as possible, and stop the spread of the infection. Report an outbreak to local Health protection team (HPT)
Chicken Pox	Approx. 5 days from the onset of the rash	Blisters on rash must be dry and crusted over before coming back
Cold sores	None,	Avoid kissing and contact with sores.
conjunctivitis	none	If an outburst /cluster occurs, consult local Health protection team (HPT)
Diphtheria*	Is essential to keep away, always consult with local HPT	All family contact must stay away from the setting until cleared by the HPT. Preventable by vaccination.
Diarrhoea and Vomiting	48 hours after symptoms cleared	Seek information from HPT if unsure
E. Coli,0157 VTEC Typhoid* [and paratyphoid*] (enteric fever) Shigella (dysentery)	Should be excluded for 48 hrs, from last episode of Diarrhoea.	Further exclusion maybe required, (until no longer excreting)
Glandular Fever	none	
Head lice	none	Treatment is recommended, only when live lice seen

INFECTION	RECOMMEND PERIOD OF TO BE KEPT AWAY	COMMENTS
Salmonella and Dysentery	Until certified well by doctor	
Hand, Foot and Mouth disease	none	Contact HPT if large number of children are affected.
Hepatitis A*	Exclude until 7 days after the onset of jaundice (or 7 days after symptoms onset if no jaundice)	Your local HPT will
Hepatitis B* and C* HIV/AIDS	None	Hepatitis B and C and HIV are bloodborne viruses not infectious through casual contact.
Impetigo	Until the skin has healed (or after 48hours after commencing antibiotic	Antibiotic speeds healing and reduces the infectious period
Influenza	Until recovered	Report an outbreak to the HPT
Measles*	4 days from onset of rash	Preventable by vaccination, promote MMR for all children and staff, pregnant staff to contact GP or midwife
Meningitis*due to other bacteria	Until recovered	Preventable via vaccine, local PHE centre will give advice on any action to take
Meningitis* viral	none	
Meningococcal meningitis*/septicaemia*	Until recovered	Preventable by vaccination
Mumps*	5 days from onset of swelling	Preventable by vaccination
MRSA	none	Good hygiene needed, in particular handwashing and environmental cleaning.
Monkey Pox	21 days from the onset	begins with fever, headache, muscle aches, backache, swollen lymph nodes, a general feeling of discomfort, and

		exhaustion. 1 to 3 days after there maybe a fever, develop a papular rash (i.e., raised fluid-filled bumps), often first on the face but sometimes initially on other parts of the body, especially the genital and perianal areas. The lesions usually develop through several stages before crusting and falling off over the course of 2-4 weeks
Pertussis* (Whooping cough)	2 days from starting antibiotics, or 21 days from the onset or 5 days from commencing antibiotic treatment	
Ringworm	Exclusion not normally required	Treatment is required
Rubella* (German Measles)	4 days from onset of the rash	Preventable by vaccination, promote to children, parents and staff.
Scabies	Can return after first treatment	Household and close contacts require treatment
Scarlet fever*	Child can return 24 hours after starting the appropriate antibiotic treatment	A person is infectious for 2-3 weeks if antibiotics are not administered. If 2 or 3 cases are known, see advice from HPT
Slapped Cheek/fifth disease. Parvovirus B19	None (once the rash has developed)	Pregnant lady to seek medical advice
Streptococcal infection of the throat	3 days from the start of the treatment	
Threadworms	None	Treatment recommended for family
Tonsillitis	None	
Tuberculosis*	ALWAYS consult HPT BEFORE disseminating info to staff and parents	Only pulmonary (lung) TB is infectious to others, needs close prolonged contact to spread.
Warts (including Verruca) None.	none	Should be covered in hall, changing rooms and swimming

* Denotes a notifiable disease. Contact will be made from persons GP to communicable Disease control

If there is an outbreak of food poisoning of 2 or more children we will inform the appropriate authorities within the food standards agencies and Ofsted.

Medication – children and staff

The owner/manager (Mandy Grosse) or deputy manager (Natalie Walton) will discuss the necessity of the child receiving the medication at the Club with the parents/carers. Medicines should only be taken to the Club when it is essential. **The Owner/manager** will have the final responsibility for accepting the care of the child. Parents/carers should respect that decision as it will be taken with an awareness of the health and safety needs of the other children attending the club. It is for the Owner to arrange who should administer medicines, either on a voluntary basis or as part of a contract of employment.

Full instructions should be given to the Owner in writing. All medicines must be prescribed by a doctor, dentist, nurse or pharmacist, with the exception of paracetamol or calpol (painkillers). The staff do appreciate that in some cases all a child needs is some medication for pain to help them get through a session. Such as period pains in older girls, or if they have a raised temperature. The staff have agreed, that as long as parents give permission to administer this type of medication, over a period of no longer than 24 hours, we will. If, however the pain or symptoms still persist after 24 hours, we will no longer agree to administer the medication until they have been checked over by a doctor, dentist, nurse or pharmacist. Forms will be completed just as they are for prescribed medication, with times of last dosage given.

The medication must be labelled clearly with the child's name and stored as per the instructions.

Medicines should be kept in locked non-portable container (except where storage in a fridge is required) and only named individuals should have access. A record will be kept for audit and safety purposes.

Medicines should be administered by the person in charge of the session only and in a quiet location. The child will be observed for any untoward events following the dose given.

The times, frequency and amount of the dose must be recorded on a medicine form and this entry should be signed and dated by the parent and the person in charge of the session and put into the child's personal file for evidence.

The Owner or deputy Manager will ask parents about the medicines that their child needs to take and provide details of any changes to the prescription or the support required.

If the administration of prescription medicines requires technical/ medical knowledge then individual training should be provided for staff from a qualified

and appropriate health professional. Training should be specific to the individual child concerned, before they accept the child in club.

The owner will ensure that they have sufficient information about the medical condition of any child with long term medical needs and discuss this with the child's key person and staff that need to know.

In cases where a child usually 'self-medicates' (e.g. asthma inhalers) this will only be allowed when the club has received written permission from the parent or carer. The child in question will be asked to inform the person in charge of the session when he/she is going to take their medication and this will be recorded in their file after they observe the child taking the medication. The child's parent will be asked to sign the form and will be given the time when the child last self-medicated.

Staff taking medication

If staff are taking any medication, they have to ensure, if they are working that this medication doesn't affect their ability to care for the children, and a conversation will be had with the staff member and the manager or deputy manager, regarding this before the staff member is allowed to continue to work with the children. Any medication that the staff bring in to club, **MUST** be securely locked away and out of reach of the children **AT ALL TIMES**. Staff need to complete the same administration forms as the children do, to ensure that if there is ever an incident, and the staff member is taken to hospital, another staff member will have all of the details to hand to what the staff have taken and when.

If regular medication is taken (out of working hours) these too, should be recorded on the staff's personal file, just in case they have to attend hospital.

Signed:

Role: Owner

Review date: May 2026

Print Name: Amanda grosse

Date: April 2025

I the undersigned, am signing to say that I have reviewed this policy with the team, and will adhere to the policy at all times.

A. Grosse:

Date:

N. Walton:

Date:

H. Bailey:

Date:

T. Quinn:

Date:

Policy and procedures for extra time out of school club reviewed: April 2025

M.Wright:

Date: